

3rd week Monday case scenario

1-dr.

2-dr.

3-dr.

4-dr.

CASE 37

Mrs Intisar is a 35-year-old unemployed primigravida, who lives with her 44-year-old husband, who is also unemployed. She presents for booking at 20 weeks gestation. Her BMI is 16; she smokes 10 cigarettes per day. Her past medical history includes epilepsy; she is stable on Carbamazepine and has been seizure free for over a year. Initial booking blood tests reveal mild iron deficiency anaemia, but are otherwise normal.

37.1. Had Mrs Intisar consulted you for preconceptual counselling regarding her epilepsy, what advice would you have given her on her medication, and on risk reduction in pregnancy?

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37.2. Discuss the potential impact of low socio-economic status and other items in Mrs Intisar's history, on her pregnancy.

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37.3. Apart from routine antenatal care, what additional tests do you think this patient should have during the course of her pregnancy?

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Mrs Intisar is found to be anaemic on her booking bloods with a HB of 8.2gm/dl.

37.4. What are the clinical consequences of anaemia in pregnancy, labour and purperium?

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37.5. What investigations would you perform to diagnose the type of Mrs Intisar's anaemia?

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Mrs Intisar contacts her midwife at 24 weeks gestation. She is too distressed to talk on the phone, and the midwife therefore arranges a home visit. She finds Mrs Intisar with bruises on her face and her arm in a sling. It appears that her husband has been arrested for assaulting her.

37.6. Review the following publication: Domestic violence in pregnancy. Mezey&Bewley, BMJ 1997;314:1295. You can access an electronic copy on the BMJ website (www.bmj.com).

37.7. Having read the article, please outline the impact of domestic violence on pregnancy.

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Mrs Intisar's husband is remanded in custody, and she is now unsupported as she does not have family or friends locally. At 32 weeks gestation her neighbour calls an ambulance, as he finds her collapsed outside her front door. The actual collapse was unwitnessed. She is brought to hospital by ambulance. The admitting team assume that she had an epileptic seizure. On arrival she is more alert, and complains of a headache and epigastric pain. Her BP is 166/100 mm Hg.

37.8. What would be your main differential diagnosis?

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37.9. What investigations would you request to confirm or refute your suspected diagnosis?

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Whilst you are assessing Mrs Intisar, you note that her BP has risen to 170/124 mm Hg, and you are more certain of the diagnosis. The fetal heart is normal.

37.10. Describe how you would manage the patient and why.

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37.11. Describe methods of induction of labour and the relevance of the Bishop Score

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37.12.What are potential maternal and fetal complications of the condition this patient has?

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37.13. Describe three different drugs from three different therapeutic groups that may be used to treat hypertension in pregnancy, including mode of action, usual dosage regimens, routes of administration, contraindications and side effects.

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